



Maura Healey, Governor
Kimberley Driscoll, Lieutenant Governor
Monica Tibbitts-Nutt, Secretary & CEO
Phillip Eng, General Manager & CEO



Honorable Michael J. Rodrigues
Chair, Senate Committee on Ways & Means
State House, Room 212

Honorable Aaron Michlewitz
Chair, House Committee on Ways & Means
State House, Room 243

Honorable Brendan Crighton
Chair, Joint Committee on Transportation
State House, Room 109-C

Honorable James Arciero
Chair, Joint Committee on Transportation
State House, Room 134

Honorable Mark C. Montigny
Chair, Senate Committee on Post Audit and
Oversight
State House, Room 312-C

Honorable John J. Mahoney
Chair, House Committee on Post Audit and
Oversight
State House, Room 146

Honorable John C. Velis
Chair, Joint Committee on Mental Health,
Substance Use and Recovery
State House, Room 513

Honorable Mindy Domb
Chair, Joint Committee on Mental Health,
Substance Use and Recovery
State House, Room 33

Michael Hurley, Senate Clerk
Office of the Clerk of the Senate
State House, Room 335

Timothy Carroll, House Clerk
Office of the Clerk of the House
State House, Room 145

October 8, 2025,

Dear Chairs Rodrigues, Michlewitz, Crighton, Arciero, Montigny, Mahoney, Velis and Domb,

Pursuant to Line Item 1595-6369 of Chapter 9 of the Acts of 2025, the MBTA submits the following report and accounting for the funds provided in Line Item 4512-0206 of Chapter 28 of the Acts of 2023 regarding the placement of naloxone in red line stations for public access.

MBTA's Naloxone (Narcan)¹ Pilot Distribution Program

In August 2024 the MBTA began a pilot program to make naloxone available to the public in select Red Line stations. This program was funded via legislative appropriation, and naloxone was purchased through partnership with the Department of Public Health. The MBTA installed distribution cabinets and made public awareness messaging available in multiple languages. This report summarizes the MBTA's activities and use of the appropriated funds through the end of FY2025. Members of the public took naloxone from the cabinets, but the MBTA is unaware of any instances of the provided naloxone having been used on site at the pilot stations. The MBTA appreciates the challenges of substance abuse and overdose in the community and will continue to ensure that all members of the Transit Police Department have naloxone and are properly trained on its usage. However, given the challenges of temperature control for storage of naloxone and the staffing

¹ Naloxone is the generic name for Narcan, an opioid overdose reversal medication. The terms may be used interchangeably through this report.

impacts of the restocking of the product across the geographic spread of subway stations, the MBTA recommends that a third party take responsibility for monitoring, staffing and refilling naloxone within the MBTA's public transit stations. The MBTA has begun preliminary conversations with those in the public health field and would welcome support of their expertise to ensure a successful transition and continuation of the program.

Report and Accounting of MBTA Public-Access Naloxone Pilot

1. Which Red Line stations have accessible naloxone

Beginning July 31, 2024, the MBTA installed 15 cabinets containing naloxone for public access at five pilot stations: Quincy Center, Ashmont, Andrew, South Station, and Harvard. At each station, cabinets were installed in the unpaid and paid lobby areas as well as near or inside publicly accessible station restrooms. The pilot stations were selected for high ridership, their proximity to available assistance from the Transit Police Department in the event of an emergency, and because these stations offer a more temperature-controlled environment which is required to store naloxone over extended periods of time. Signs in English, Spanish, and Chinese were posted with each cabinet with information about how to recognize an opioid overdose, how to administer naloxone, and how to contact emergency assistance.

2. Program expenditures

The FY2024 state budget allocated \$95,000.00 to the MBTA to establish the program. At the end of FY2024, the funds were rolled over for use through FY2025. The MBTA's access to the funds expired at the close of FY2025. During the pilot, the MBTA expended \$55,515.54 as follows:

Item	Quantity	Total Cost
Cabinets	90	\$28,760.40
Transit Facilities Maintenance Labor Costs	Installation of 15 cabinets	\$11,337.46
Narcan (6/24/24)	72	\$2,240.64
Narcan (7/8/24)	72	\$2,240.64
Narcan (9/6/24)	132	\$4,107.84
Narcan (11/18/24)	72	\$1,968.48
Narcan (1/10/25)	72	\$1,968.48
Narcan (4/15/25)	96	\$2,399.04
Station Supplies for recording replacement data		\$492.56
	TOTAL	\$55,515.54

3. The dates when each of the pilot red line stations received naloxone

The MBTA received shipments of naloxone to JFK station for distribution to the pilot stations. Naloxone was delivered to JFK station on:

- June 28, 2024
- July 12, 2024
- September 12, 2024
- November 22, 2024
- January 16, 2025
- April 22, 2025

After being received at JFK station, naloxone was transferred to the pilot stations based on reported need.

4. Record Keeping, uptake and replacement rates

The MBTA replaced the naloxone as needed by station officials. In addition, the MBTA notes the following:

- Cabinets were checked daily and any missing naloxone was replaced; cabinets were consistently stocked with naloxone.
- No cabinets have been reported damaged.
- There were no known instances of on-site use of naloxone from the cabinets or overdoses that can be linked to the public-access naloxone. As far as the MBTA is aware, all doses taken from the cabinets have been carried off site.
- The highest requests from MBTA staff for replacement inventory have been for Quincy Center, Andrew and South Station.

5. Future of the program

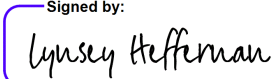
During the pilot, the MBTA utilized existing red line operations personnel perform daily checks, record-keeping and replacing naloxone in the cabinets. Inventory distribution and data collection added additional challenges to MBTA operations staffing; the MBTA does not have an existing job classification suitable for long-term implementation of a program of this nature across a subway line, and many Red Line stations are not continuously staffed with dedicated personnel. During the pilot year, administrative staff at the MBTA's Park Plaza headquarters undertook the additional duty of ordering naloxone and personally delivering it to a central location on the Red Line for distribution, but this procedure is not sustainable. Moreover, the MBTA selected the five pilot stations based on the ability to store the naloxone in a relatively temperature-controlled environment, and the logistics of installing electronic, temperature-controlled boxes and/or replacing naloxone on a more frequent cadence during extreme heat or cold remain unknown.

The MBTA recognizes and supports the importance of promoting harm reduction efforts and increasing public access to lifesaving medication that can reverse an opioid overdose. As a result of this pilot, the MBTA has determined that it does not currently have sufficient resources or appropriate personnel needed to manage the day-to-day administration of such a program. However, MBTA Transit Police Officers will continue to carry naloxone.

The MBTA recommends that any public access program in or near stations be managed by a third party such as an agency whose mission is aligned with such an important program and that has the resources and personnel to monitor and resupply naloxone as well as to manage for temperature fluctuations. The MBTA has conducted outreach to the Department of Public Health Bureau of Substance Addiction Services and to Cambridge Health Alliance seeking assistance from these organizations, including suggestions for another public health partner suitable to assume responsibility for the program. To date, no partner has been identified. In the event staffing and program management can be made available from a third party, the MBTA can continue to make space available in its facilities to support the program.

Sincerely,

Signed by:



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Lynsey M. Heffernan
Chief of Policy & Strategic Planning